

Medication & Infusion Cheat Sheet

IV protocols of record · USP 797 prep · RN clinical reference

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INTERNAL — CLINICAL STAFF ONLY (NO PATIENT PRICING) · 2026-06-29

IV MYERS COCKTAIL ADMINISTRATION

Protocol slug: iv-myers-cocktail · sourced from 20260509140000_seed_clinical_protocol_drafts.sql

Indication	Adult IV wellness / nutrient repletion per standing orders.
Medication	Myers cocktail components as listed
Dose	Per formulation above
Base / diluent	Magnesium chloride 1g, calcium gluconate 100mg, B-complex 1mL, B12 1mg, B5 250mg, B6 100mg, vitamin C 5g, in 250mL normal saline.
Route	IV infusion
Frequency	Per order / membership benefit
Duration	30–45 minutes
Pre-administration checks	Verify identity · Two RN checks on bag labeling · Patent IV access
Monitoring (during)	VS q15min first 30min · Patient comfort
Monitoring (post)	Discharge instructions · Adverse event reporting pathway
Administration	Prime line · Infuse per rate policy · Dispose sharps per clinic SOP

OPEN reviewer note: Verify Myers formulation matches Henry Schein standard pre-mixed bag if you're using one, vs. compounded in-house

OPEN reviewer note: Confirm 30–45 min infusion rate matches your preference; some clinicians push slower for first-time patients

OPEN reviewer note: Vitamin C dose of 5g — verify G6PD screening expectation in your intake (high-dose IV C is contraindicated in G6PD deficiency)

IV NAD+ 250MG INFUSION

Protocol slug: iv-nad-250mg · sourced from 20260509140000_seed_clinical_protocol_drafts.sql

Indication	NAD+ IV therapy 250mg session.
Medication	NAD+
Dose	250mg
Base / diluent	NAD+ 250mg in 500mL normal saline.
Route	IV infusion
Frequency	Per order
Duration	60–90 min
Pre-administration checks	Baseline VS · Pregnancy status if applicable
Monitoring (during)	VS per IV lounge policy · Symptom assessment q15–30min
Monitoring (post)	Post-infusion check · Home instructions
Administration	Dilute per pharmacy label · Gradual rate titration per tolerance

OPEN reviewer note: Verify NAD source — FCC compounded vs. commercial. Affects concentration and dilution math

OPEN reviewer note: Some clinicians use Glutathione push at end of NAD+ — confirm if you want this in standard protocol or as add-on

OPEN reviewer note: First-time patient slower start (90–120 min) — confirm threshold for 'first-time'

IV NAD+ 500MG INFUSION

Protocol slug: iv-nad-500mg · sourced from 20260509140000_seed_clinical_protocol_drafts.sql

Indication	NAD+ IV therapy 500mg session.
Medication	NAD+
Dose	500mg
Base / diluent	NAD+ 500mg in 500mL normal saline.
Route	IV infusion
Frequency	Per order
Duration	90–120 min
Pre-administration checks	Review prior NAD+ sessions · VS baseline
Monitoring (during)	VS q15min early · Symptom log
Monitoring (post)	Extended observation if symptoms
Administration	Longer observation window · Gradual titration

OPEN reviewer note: Confirm step-up requirement — should patients always do 250mg first, or is 500mg first-time acceptable for low-risk patients?

OPEN reviewer note: Verify max dose ceiling — some clinicians cap at 500mg per session; others go to 750mg or 1g

IV GLUTATHIONE PUSH

Protocol slug: iv-glutathione-push · sourced from 20260509140000_seed_clinical_protocol_drafts.sql

Indication	IV glutathione push.
Medication	Glutathione
Dose	1–2g per order
Base / diluent	Glutathione 1–2g IV push.
Route	IV push
Frequency	Per order
Duration	5–10 min
Pre-administration checks	Verify dose on order · Patent IV
Monitoring (during)	Continuous observation
Monitoring (post)	Post-push VS
Administration	Slow push · RN at bedside

OPEN reviewer note: Confirm dose preference — 1g standard, 2g for higher-need patients, or always 2g?

OPEN reviewer note: Some clinicians require sulfa allergy screening (theoretical cross-reactivity, debated). Confirm your stance

Prescription Injectables — NO PROTOCOL OF RECORD

Do not administer Toradol, Zofran, Benadryl, or Pepcid until a clinical protocol is signed in the Protocol Library.

Agent	Informal use	Status
Toradol (ketorolac)	Anti-inflammatory IV push	⊖ NO PROTOCOL — do not administer until signed
Zofran (ondansetron)	Antiemetic IV push	⊖ NO PROTOCOL — do not administer until signed
Benadryl (diphenhydramine)	Antihistamine IV/IM	⊖ NO PROTOCOL — do not administer until signed
Pepcid (famotidine)	H2 blocker IV	⊖ NO PROTOCOL — do not administer until signed

PREPARATION / RECONSTITUTION / ADMIXTURE (USP 797)**Before you touch anything**

1. Perform hand hygiene (soap + water or ABHR) before entering the prep area and after removing gloves.
2. Confirm ****patient identity****, ****signed order****, and ****allergy status**** against the chart and wristband.
3. Inspect every vial, bag, and ampule: correct drug, concentration, beyond-use date (BUD), intact seal, no particulate or discoloration.
****Do not use**** compromised containers.
4. Gather only the supplies needed for this single patient preparation (ISO 5 / PEC mindset even in a non-compounding primary suite — minimize traffic and clutter).

Work surface & garbing

- Disinfect the prep counter with approved sporicidal/low-level disinfectant; allow full contact time per manufacturer label.
- Gloves required for all manipulations. Mask and hair cover when preparing multiple admixtures in one session or when pharmacy SOP requires.
- No food, drink, or personal phones on the prep surface.

Reconstitution (peptides / lyophilized vials)

- Use **bacteriostatic water for injection (BWFI)** or diluent specified on the **pharmacy label** — never swap diluents without pharmacist or physician direction.
- Inject diluent slowly down the vial wall; gentle swirl only — **do not shake** unless label instructs.
- Document lot number, reconstitution date/time, and initials on the patient-specific label.
- Apply BUD from pharmacy label or USP 795/797 tables — when in doubt, use the **shorter** dating. Refrigerate if label requires.

Admixture (IV bags & syringes)

- Use **closed-system transfer device (CSTD)** or needleless connectors when available.
- Add medications to the bag **in order listed on the protocol**; invert gently after each addition — do not shake protein/peptide-containing solutions unless protocol specifies.
- Label every final container with: patient name, drug(s), concentration, total volume, route, BUD date/time, preparer initials.
- **Two-RN verification** required for high-alert additions (electrolytes, insulin-class, chemotherapy-class, or per clinic policy) and for all bags leaving the prep area.

Immediate-use vs. stored preparations

- **Immediate-use** (administer within 1 hour of prep, no BUD storage): permitted when no high-risk compounding and patient is on-site — still label and document.
- **Stored admixtures** (refrigerated or room temp per USP): only per written pharmacy policy with assigned BUD; log in compounding record.

Administration handoff

- Compare the prepared product to the **signed clinical protocol** (or physician order if protocol not yet signed).
- Prime IV tubing per manufacturer; verify infusion rate against protocol duration.
- Remain at bedside for IV pushes and for the first 15 minutes of any new infusion per IV lounge policy.

Waste & documentation

- Discard partial vials and sharps per clinic SOP and Georgia medical waste rules.

- Record lot numbers, prep time, administrator, and any adverse events in the IV flowsheet / EHR.

When to stop and escalate

- Missing or unsigned protocol for the medication you are about to give → **hold** and notify physician.
- Patient fails pre-administration checks on the protocol → **do not administer**; notify RN lead or physician.
- Any break in aseptic technique → discard prepared product and start over.